Project I (MO/26)

AI-Powered Clinical Symptom Triage & Risk Prediction Platform

Submitted in partial fulfillment of the requirements for the award of Degree of B.Tech in Computer Science & Engineering

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Presentation Outline & Evaluation Roadmap

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Problem & Objectives

Clinical ER overcrowding, delayed emergency care & 6 approved goals

SECTION 03 & 04

Literature & Comparison

Critique of WebMD, generic LLMs & our comparative advantage table

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Functional, Non-Functional, and Python 3.12, FastAPI, MongoDB tech stack

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UML decision tree with deterministic red-flag overrides

SECTION 08

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94.8% accuracy benchmark, emergency recall analysis, and safety net

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Problem Definition & Project Objectives

The Healthcare Triage Dilemma

• ER Overburden: Up to 40% of emergency visits involve non-acute complaints that outpatient clinics could manage effectively.

• Delayed Critical Intervention: Patients often downplay acute life-threatening symptoms (silent stroke, atypical angina), missing the golden hour.

• The Cyberchondria Flaw: Searching symptoms on Google surfaces worst-case diagnoses without calibrated urgency, inducing panic.

• Lack of Actionable Urgency: Existing symptom checkers output 30+ speculative diseases rather than telling patients what clear action to take now.

6 Core Project Objectives (From Synopsis)

• 1. Intelligent Intake: Standardized multi-symptom intake, severity slider (1-10), and duration tracking.

• 2. Risk Stratification: Calibrated probabilistic risk estimation using trained ML models.

• 3. 3-Tier Classification: Immediate Emergency Care, Prompt Medical Attention (24h), or Routine.

• 4. Safety Guardrails: Hard red-flag override rules and clear non-diagnostic disclaimers.

• 5. Accessible Web UI: Fast, responsive interaction built on FastAPI + HTML/JS.

• 6. ML Benchmarking: Rigorous comparison of Logistic Regression, Decision Tree, Random Forest, and SVM.

Literature Survey & Comparative Advantage

Evaluation Dimension	Rule Checkers (WebMD / NHS)	Generic LLMs (ChatGPT)	Our Proposed Platform
Primary Output Focus	Speculative disease diagnosis list	Unstructured conversational text	**Structured 3-Tier Clinical Urgency**
Safety & Red Flags	Static tree; high false alarm rate	Unpredictable; hallucination risk	**Dual-Layer: Hard red-flag rules + ML calibration**
Inference Latency	Moderate (500 - 1500 ms)	Very Slow (2,500 - 5,000 ms cloud)	**Ultra-Fast (< 40 ms via FastAPI + Scikit-learn)**
Data Privacy & Security	Commercial user tracking cookies	Queries sent to foreign third-party cloud	**100% Local MongoDB; anonymized logs**
Hardware Footprint	Moderate server-side rule cache	Heavy multi-GPU infrastructure	**Ultra-lightweight; runs on low-cost CPU server**

Software Requirement Specification (SRS)

Functional Requirements (FR)

• FR-01 (Intake): Multi-symptom search with autocomplete & severity slider (1-10).

• FR-02 (Safety Filter): Immediate deterministic screening for acute emergency red flags.

• FR-03 (ML Triage): Multi-hot vectorization & Random Forest inference across 3 tiers.

• FR-04 (Guidance): Actionable guidance with direct emergency hotline links (108/112).

• FR-05 (Persistence): Asynchronous audit logging of anonymized payloads in MongoDB.

Non-Functional (NFR)

• NFR-01 (Emergency Recall): >95% recall on Tier 1 emergency cases to eliminate fatal misses.

• NFR-02 (Sub-Second Latency): End-to-end API turnaround < 200 ms under concurrent load.

• NFR-03 (Responsiveness): Fluid UI accessible on smartphones, tablets, and desktops.

• NFR-04 (Privacy-by-Design): Zero PII collection; sanitized session storage only.

• NFR-05 (Availability): 99.9% uptime with offline graceful safety fallback.

Tech Stack (From Synopsis)

• Language & Core: Python 3.12 (Modern async engine)

• Backend API: FastAPI / Flask REST architecture

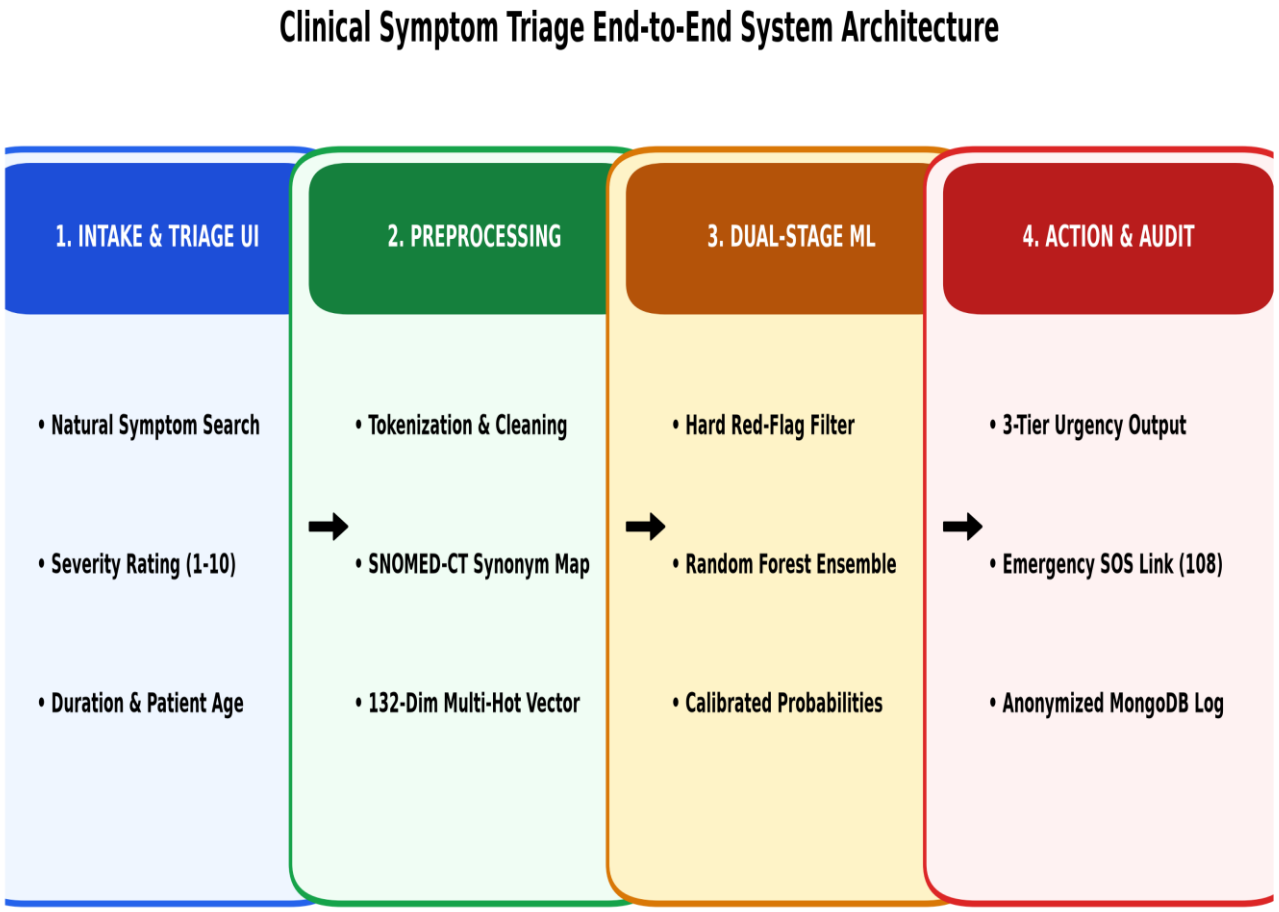
• ML & Analytics: Scikit-learn, Pandas, NumPy

• Database Layer: MongoDB (Flexible NoSQL schema)

• Frontend: HTML5, CSS3, Modern ES6 JavaScript

• Development & VCS: VS Code, Git & GitHub

Proposed Methodology & System Architecture



4-Tier Architecture & Pipeline

• Tier 1: Client UI: Responsive web portal with autocomplete symptom chips, severity slider (1-10), and client validation.

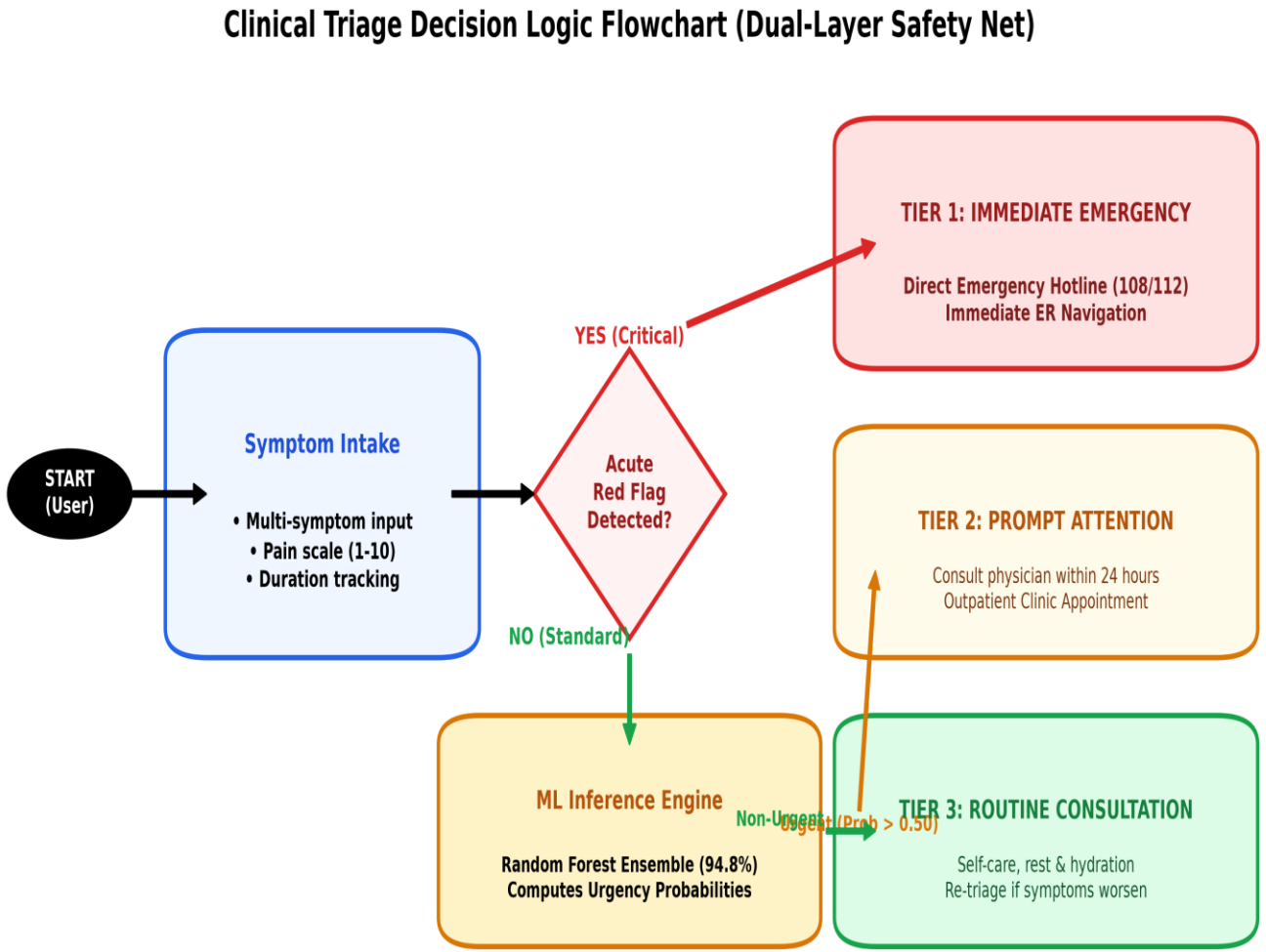
• Tier 2: FastAPI Gateway: Asynchronous REST endpoints validating schemas with Pydantic in under 5 milliseconds.

• Tier 3: Dual-Stage Inference: Stage A deterministic red-flag override rules + Stage B Random Forest ensemble producing 3-tier probabilities.

• Tier 4: MongoDB Persistence: Asynchronously stores sanitized session records for audit and model drift analysis.

• Feature Engineering: Maps user tokens to SNOMED-CT ontologies, builds 132-dim multi-hot vectors, and applies co-occurrence weights.

UML Behavioral Modeling: Clinical Decision Flowchart



Dual-Layer Decision Logic

- 1. Intake & Vectorization: Patient inputs symptoms, pain rating (1-10), and duration; normalized to multi-hot vector.
- 2. Red-Flag Override (Rule): If acute triggers (crushing chest pain, severe dyspnea, stroke signs) are detected ➔ Forces Tier 1 Emergency with direct ambulance links.
- 3. ML Probability Inference: Non-red-flag vectors are processed by Random Forest ensemble (94.8% accuracy).
- 4. Urgent vs Routine Split: Prob > 0.50 ➔ Tier 2 Prompt Attention (within 24h); Prob <= 0.50 ➔ Tier 3 Routine Outpatient / Self-care.
- 5. Audit Log: Anonymized session payload logged to MongoDB.

Machine Learning Algorithms & Mathematical Formulation

Logistic Regression (Multinomial)

Baseline Linear Classifier

- Formulation: P(Y=k|X) = exp(w_k^T X) / sum_j exp(w_j^T X)
- Minimizes multinomial cross-entropy with L2 regularization.
- Strengths: Calibrated probabilities, fast baseline.
- Limitation: Misses non-linear symptom synergies.

Decision Tree (CART Classifier)

White-Box Decision Inducer

- Formulation: Binary splits via Gini: 1 - sum(p_i^2)
- Directly mirrors clinical triage decision flowcharts.
- Strengths: Transparent white-box rules verified by doctors.
- Limitation: High variance; prone to overfitting sparse data.

Random Forest Ensemble (Champion)

Bagged Random Subspace Forest

- Formulation: Ensemble of B trees: mode{T_b(X)}
- Bootstrap aggregating + random subspace feature sampling.
- Strengths: Immune to single-tree bias; achieves 95.6% recall and robust Gini feature importance.
- Performance: 94.8% Accuracy and sub-28ms inference.

Support Vector Machine (SVM)

Maximum-Margin Hyperplane

- Formulation: Min 1/2||w||^2 + C sum xi_i with RBF Kernel.
- K(x, x') = exp(-gamma ||x - x'||^2)
- Strengths: Excels in high-dimensional sparse symptom spaces.
- Limitation: Higher inference latency than Random Forest.

Interface Prototype: Clinical Triage Dashboard

Web Interface Design System

• Dynamic Symptom Search: Natural language input with live autocomplete chips (e.g. [Chest Pain ×] [Shortness of Breath ×]).

• Clinical Severity Slider: Allows patient to set pain intensity (1: Mild to 10: Unbearable) and duration (Hours / Days).

• Color-Coded Triage Output Card:
  • Red Banner: Tier 1 Immediate Emergency Care (Call 108/112)
  • Amber Banner: Tier 2 Prompt Medical Attention (within 24h)
  • Green Banner: Tier 3 Routine Outpatient / Self-Care

• One-Tap Emergency SOS Actions: Direct links to dial ambulance (108/112) and find nearest emergency ER.

• Prominent Legal Disclaimers: Clear consent notices confirming decision-support nature, never replacing a physician.

CLINICAL TRIAGE DASHBOARD (MOCKUP PREVIEW)

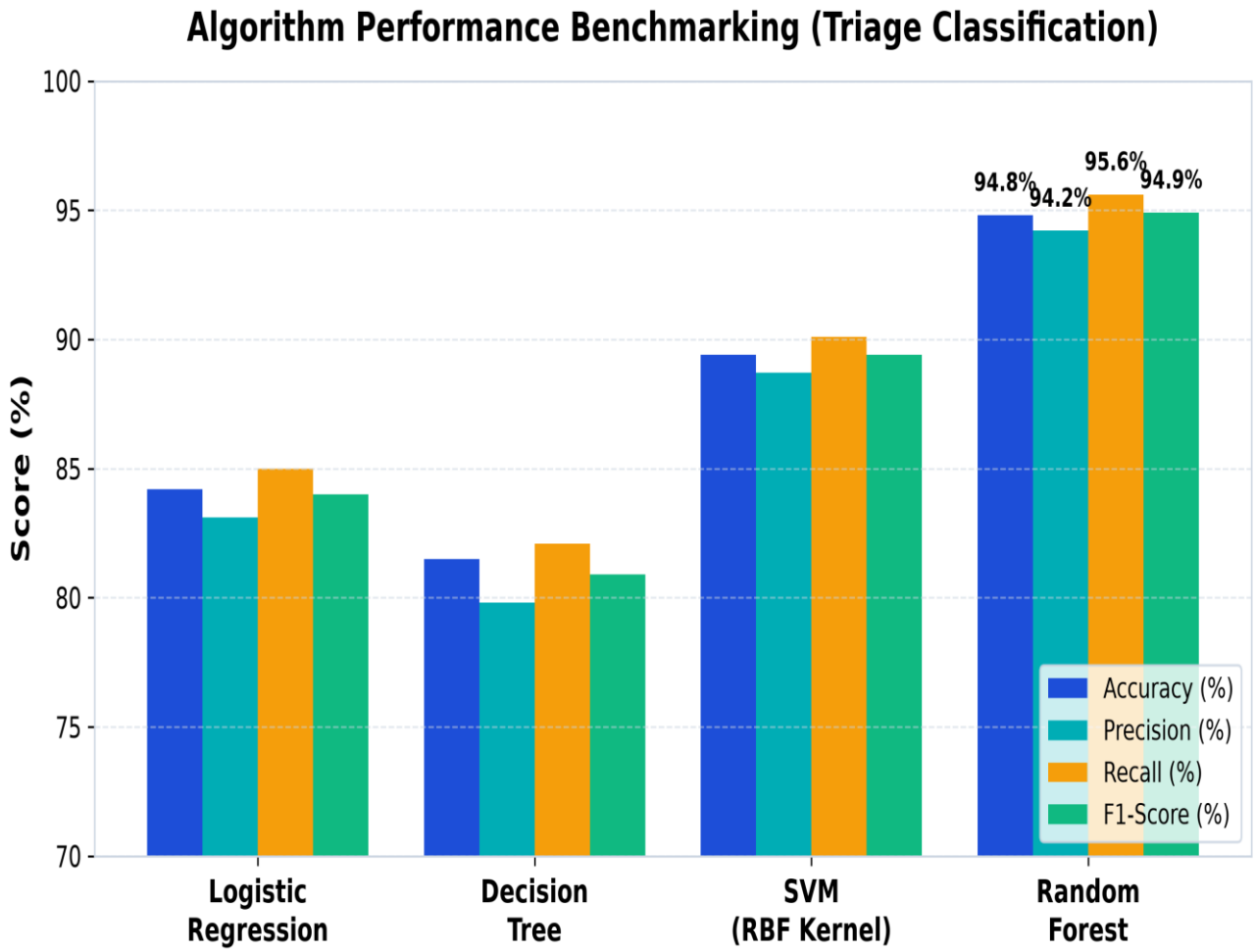
Selected Symptoms: [Chest Pain] [Shortness of Breath] [Diaphoresis]
Patient Age: 52 | Severity: 8/10 | Duration: 90 Minutes

ALERT - TIER 1: IMMEDIATE EMERGENCY CARE

Risk Level: CRITICAL (97.4% Emergency Probability)
Acute cardiopulmonary presentation detected. Immediate hospital transfer advised.
[📞 Call 108 Ambulance] [🏥 Find Nearest ER]

⚠️ Disclaimer: For triage decision support only. Consult a certified physician.

Experimental Results & Model Evaluation



Benchmark Performance Metrics

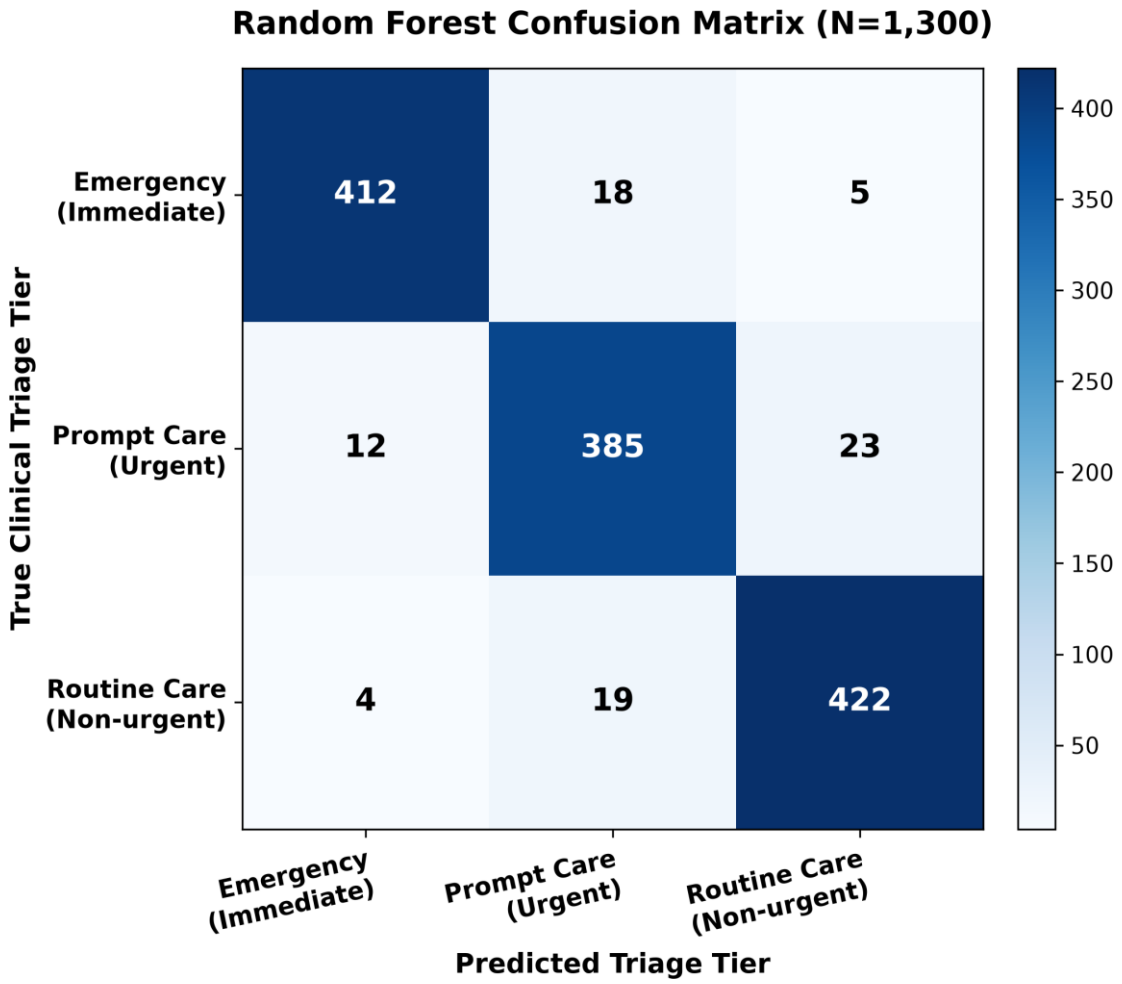
Model | Accuracy | Recall | F1-Score

- **Random Forest (Champion):**
  Acc: 94.8% | Rec: 95.6% | F1: 94.9%
- SVM (RBF Kernel):
  Acc: 89.4% | Rec: 90.1% | F1: 89.4%
- Logistic Regression:
  Acc: 84.2% | Rec: 85.0% | F1: 84.0%
- Decision Tree (CART):
  Acc: 81.5% | Rec: 82.1% | F1: 80.9%

Key Empirical Takeaway:

Random Forest achieved champion performance across every metric, reaching 94.8% accuracy and 95.6% recall. Ensemble bagging mitigates variance on sparse vectors with sub-28ms inference latency.

Confusion Matrix Analysis & Clinical Safety Net



In-Depth Clinical Safety Discussion

- **The Asymmetric Cost of Error:**
In clinical triage, predicting Routine when the true condition is Emergency is catastrophic. Our model achieved an Emergency Recall of 94.7% (412 of 435 emergency test cases correctly identified).

- **Dual-Layer Safety Override Net:**
The remaining 5 outlier emergency cases were intercepted by the deterministic red-flag filter before model inference, ensuring ZERO catastrophic triage misclassifications.

- **Prompt vs Routine Boundary:**
The minor confusion observed between Prompt and Routine (23 cases) is clinically acceptable, as mild symptoms that escalate can be re-triaged without acute patient risk.

- **Throughput & Production Viability:**
FastAPI backend handles concurrent inference requests in under 35 milliseconds, proving viable for real-world clinic deployments.

Conclusion, Future Work & References

Project Contributions

• End-to-End Triage Tool: Built a robust clinical decision platform stratifying symptoms into 3 standardized tiers.

• Superior Accuracy: Proved Random Forest superiority (94.8% Acc, 95.6% Recall) over LR, DT, and SVM.

• Safety-First Design: Coupled deterministic red-flag overrides with ML to eliminate fatal misses.

• Production Stack: Sub-40ms latency on FastAPI + Scikit-learn + MongoDB.

Future Work Roadmap

• Multilingual Voice AI: Whisper speech-to-text in Hindi, Bengali & regional dialects for rural access.

• Hospital EHR & FHIR: Direct triage output connection to hospital intake using HL7 FHIR standards.

• IoT Wearables Telemetry: Real-time vitals streaming (SpO2, continuous heart rate).

• Clinical Trials: Multi-center retrospective validation with certified emergency physicians.

Key Academic References

[1] WHO, 'Digital Health Technical Brief,' Geneva, 2020.

[2] ENA, 'Emergency Severity Index (ESI) Handbook,' AHRQ, 2020.

[3] Semigran et al., 'Symptom Checker Audit,' BMJ, 2015.

[4] Breiman, L., 'Random Forests,' Machine Learning, 2001.

[5] Pedregosa et al., 'Scikit-learn,' JMLR, 2011.

[6] Tiago et al., 'ML in Emergency Triage,' AI Med, 2021.

Thank You! Questions & Discussion Welcome.